INVESTMENT & STRATEGY MEMO

Company: CompoundIQ — Functional Medicine Infrastructure Platform **Document Type:** Venture Capital Stress-Test & Go-To-Market Strategy **Prepared For:** Founding Team / Seed Investors **Last Updated:** April 13, 2026 **Version:** 3.0

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1. Executive Summary & Core Thesis

Functional medicine is expensive, not because providers are greedy, but because the backend is broken. If a provider sees 12-20 patients a day, they aren't just doing clinical care; they are running a manual sourcing operation (compounded scripts, peptides, state compliance checks, pricing comparisons). This unbillable administrative tax kills profitability, forces patients to gray-market peptides, and causes burnout.

CompoundIQ is the unified B2B2C operational backbone that solves this: a "Compounding Pharmacy Intelligence Engine" that handles the full lifecycle — search, price, pay, route, track, and deliver — in a single platform. Today the platform is **live in production** at https://functional-medicine-infrastructure.vercel.app, validated through 6 rounds of independent QA with zero remaining findings.

The Tech Comparables (The Vision):

- **"Bloomberg Terminal"** for functional prescriptions (centralized real-time intelligence layer).
- **"Expedia"** for compounding pharmacies (compare, choose, route in one interface).
- **"Stripe"** for cash-pay medicine (invisible B2B2C backend powering multi-party fulfillment).
- **"Plaid"** for pharmacy & peptide data (aggregating fragmented systems).
- **"Shopify"** for clinics (giving independent providers enterprise-grade operational tools).
- *The VC Shorthand:* "We are building Rupa Health, but for compounded medications instead of specialty labs."

The core thesis: **Build the "Expedia + Stripe" for compounded medications.**

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2. Market Opportunity

Market Size & Growth

The compounding pharmacy market in the United States is valued at approximately **$13–15 billion annually**, with the 503A (patient-specific) segment representing the majority of prescribing volume. Functional and integrative medicine is one of the fastest-growing cash-pay verticals, driven by:

- **GLP-1/weight management boom** — Semaglutide alone drove billions in compounding revenue before FDA shortage resolution in February 2025. Even post-FDA action, weight management compounding extends to Tirzepatide, BPC-157 combinations, and Lipo-Mino formulations.
- **Bioidentical Hormone Replacement Therapy (BHRT)** — Testosterone (all salt forms: Cypionate, Enanthate, Propionate, free base across injectable, cream, gel, troche, pellet, capsule, and suppository forms), Progesterone, Estradiol, and DHEA represent the steady-state foundation of compounding demand. This category is NOT subject to the regulatory volatility of GLP-1s.
- **Peptide therapy expansion** — BPC-157, Thymosin Alpha-1, GHK-Cu, MOTS-c, SS-31, DSIP, Cerebrolysin, and 5-Amino-1MQ are prescribed for tissue repair, immune modulation, mitochondrial repair, and cognitive support. Though regulatory scrutiny is increasing (BPC-157 moved to FDA Category 2 in September 2024), the broader peptide therapy market continues growing.
- **Low-Dose Naltrexone (LDN)** — Used for autoimmune conditions, MCAS, chronic pain, and neuroinflammation. Dose range 0.5mg–4.5mg across capsule, oral solution, sublingual, and topical forms. Growing adoption is driven by functional medicine providers.

The Cash-Pay Advantage

By focusing on functional medicine, compounding, and peptides, CompoundIQ bypasses the traditional insurance/Medicare reimbursement nightmare that suffocates early-stage HealthTech companies. Every transaction is cash-pay, meaning:

- No prior authorization bottleneck
- No claims adjudication delays
- No payer denials or clawbacks
- Direct patient payment with 100% collection rates on paid orders

Total Addressable Market (TAM)

- **~7,500+ compounding pharmacies** in the US (503A and 503B combined)
- **~50,000+ functional/integrative medicine practitioners** actively prescribing compounded medications
- **Average order value:** $150–$400 per prescription fill
- **Average patient protocol:** 2–7 compounded medications (real patient protocols show 7+ compounded meds simultaneously, with 12+ across multi-phase treatment plans)
- **Platform fee opportunity:** 15% of margin spread on every transaction

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3. Product — What We've Built

Platform Status

CompoundIQ is not a pitch deck — it is a working product, deployed in production, validated by independent QA.

Milestone	Status
Platform built	**19 phases, 86 work orders (81 completed, 5 in backlog)**
All 3 applications functional	Clinic App, Ops Dashboard, Patient Checkout
QA validated	**6 rounds of independent end-to-end QA — final round zero findings**
Externally tested	Cowork agent validated all 7 demo doc parts against live app
Documentation	**15+ specification documents in docs/ + 26 documents in software factory**
POC deployed	Live on Vercel — functional-medicine-infrastructure.vercel.app
Security audit	All dependency vulnerabilities resolved (0 remaining)
EPCS 2FA compliance	**DEA 21 CFR 1311 implemented — TOTP via otplib, AES-256-GCM**
Drug interaction system	**6 seeded interaction pairs with severity coding (info/warning/critical)**
Demo credential drift	**Eliminated — daily Vercel cron + in-app reset button at /ops/demo-tools**
Business action plan	3 parallel execution tracks ready (LegitScript, Catalog Data, Compliance)

Three Applications, One Codebase

Application	Users	Purpose

Clinic App (Light mode, Desktop)	Medical Assistants, Providers, Clinic Admins	Search pharmacies, price medications, set margins, send payment links, sign prescriptions, manage drafts, browse favorites + protocols, run cascading prescription builder
Ops Dashboard (Dark mode, Desktop)	Internal operations team	Pipeline monitoring, SLA tracking, pharmacy health, fax triage, catalog management, demo tools
Patient Checkout (Mobile-first, 320–428px)	Patients (guest, no login)	SMS checkout link, clinic-branded payment page, Apple Pay / Google Pay / card, zero medication details shown (HIPAA)

The User Experience

For the Medical Assistant (30 seconds per prescription):

1. Select patient & provider — patient shipping state auto-fills
2. Use **cascading prescription builder** (V3.0 hierarchical catalog): medication → salt form → formulation → dose/frequency → pharmacy
3. **OR** click a Provider Favorite for one-click reorder, **OR** load a clinic Protocol Template (multi-medication bundle)
4. Set retail price using one-click margin multipliers (1.5x, 2x, 2.5x, 3x) — see wholesale cost (locked), platform fee, and est. clinic margin in real time. Default markup pre-fills retail price.
5. **Add & Search Another** for multi-medication sessions, or **Review & Send** to batch all prescriptions
6. Provider signs once — single signature covers all prescriptions in the session
7. For controlled substances: EPCS 2FA modal triggers automatically (QR code, 6-digit TOTP, DEA 21 CFR 1311 compliance)
8. Payment link fires automatically via SMS

Save as Draft flow: MA can save without provider present. Drafts tab visible to both clinic admin and provider. Provider signs later from a dedicated review page. Order transitions DRAFT → AWAITING_PAYMENT atomically.

For the Patient (60 seconds):

1. Receive a text: "Hi Alex, Dr. Chen at Sunrise Functional Medicine has finalized your custom prescription. Tap here to pay."
2. Tap the link — opens in phone's browser. No app, no login, no account.
3. Clinic-branded checkout: "Prescription Service — $300.00." No medication details (HIPAA).
4. Pay with Apple Pay, Google Pay, or card. One tap.
5. Confirmation with delivery timeline based on pharmacy tier.

For the Operations Team:

1. Pipeline view of every order across every clinic — columns: Order, Status, Clinic, Pharmacy / Tier, SLA, Assigned, Actions
2. Color-coded SLA deadlines with automatic Slack/PagerDuty escalation
3. Pharmacy health monitoring with circuit breaker status
4. Inbound fax triage queue
5. Catalog management with version tracking and CSV upload
6. **Demo Tools page** — self-service "Reset Demo Credentials" button + recovery instructions

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4. Core Technical Architecture

Technology Stack

Layer	Technology
Frontend	Next.js 16 (App Router), React 19, Tailwind CSS + Shadcn/UI
Backend / Database	Supabase (PostgreSQL 15+), **33 tables + 1 view, 10 enums, RLS on ALL tables**
Authentication	Supabase Auth + JWT with custom claims (app_role, clinic_id)
Payments	Stripe Connect Express — multi-party split (patient → clinic payout + platform fee)
Fax	Documo mFax REST API (HIPAA-compliant, Tier 4 fallback)
SMS	Twilio Programmable Messaging (delivery tracking, opt-out handling)
Portal Automation	Playwright (headless browser for Tier 2 pharmacy portals)
Credential Storage	Supabase Vault (AES-256-GCM encryption for all pharmacy credentials)
EPCS 2FA	otplib (TOTP RFC 6238), AES-256-GCM encryption, immutable audit log
Deployment	Vercel (serverless functions, atomic deploys, instant rollback)
Cron Scheduling	Vercel Crons — **10 scheduled jobs** (SLA, payments, adapters, daily digest, POC credential sync)
Monitoring	Sentry (PHI-scrubbed) + Slack + PagerDuty

Database Architecture (33 Tables + 1 View)

Series	Tables	Purpose
V1.0 (12)	clinics, providers, patients, pharmacies, pharmacy_state_licenses, catalog, catalog_history, orders, order_status_history, webhook_events, order_sla_deadlines, inbound_fax_queue	Core business entities
V2.0 (5)	pharmacy_api_configs, pharmacy_portal_configs, adapter_submissions, normalized_catalog, pharmacy_webhook_events	Multi-tier adapter layer
Operational (4)	sms_log, sms_templates, transfer_failures, disputes	Communications + finance
Incremental (5)	clinic_notifications, ops_alert_queue, circuit_breaker_state, sla_notifications_log, catalog_upload_history	Ops + reliability
V3.0 Hierarchical Catalog (8)	ingredients, salt_forms, dosage_forms, routes_of_administration, formulations, formulation_ingredients, pharmacy_formulations, sig_templates	Cascading prescription builder data model
Provider Speed Features (3)	provider_favorites, clinic_protocol_templates, clinic_protocol_items	Favorites + multi-medication protocols

Regulatory Compliance (5)	drug_interactions, patient_protocol_phases, phase_advancement_history, epcs_audit_log, prescription_dea_tracking	DEA EPCS + drug interactions + protocol phases
1 View	provider_prescribing_history	Per-provider Rx aggregation

Order State Machine

Every order flows through a closed state machine with exactly **23 states** and **47 valid transitions**. All transitions use atomic Compare-And-Swap (CAS) to prevent race conditions.

When an order transitions from DRAFT to AWAITING_PAYMENT, **7 snapshot fields** are permanently frozen by a PostgreSQL trigger (`prevent_snapshot_mutation()`). There is no unlock mechanism — to change a locked field, the order must be cancelled and a new one created. This ensures financial auditability.

- **Terminal states:** CANCELLED, REFUNDED, DELIVERED
- **Error states:** ERROR_PAYMENT_FAILED, ERROR_COMPLIANCE_HOLD
- **SLA enforcement:** 10 SLA types with 3-tier escalation (automated → Slack → PagerDuty)

The 4-Tier Pharmacy Adapter Layer — Our Core Technical Differentiator

The Pharmacy Adapter Layer is what makes CompoundIQ possible. It abstracts the fragmented pharmacy landscape behind a single interface:

Tier	Method	Target Pharmacies	Speed	Market Coverage
Tier 1	Direct REST API	ReviveRX, Vios, MediVera, LifeFile network (Empower, Belmar, UCP, Strive), Precision	Instant (~2s)	~25%
Tier 2	Automated Portal Submission (Playwright)	Olympia/DrScript, Wells/WellsPx3	~5 min	~20%
Tier 3	Standardized API Specification (published spec)	Future partners adopting our published spec	Instant	Future 30%+
Tier 4	Fax Fallback (Documo)	Any pharmacy without Tier 1/2/3 capability	~30 min	Universal

Key architectural insights:

- **LifeFile is the single highest-leverage integration in the space.** LifeFile/ePowerRx is the de facto B2B standard for compounding pharmacies. A single LifeFile API integration unlocks Empower Pharmacy, Belmar Pharmacy, University Compounding Pharmacy (UCP), and Strive Pharmacy — the largest pharmacy network in the country.
- **Deterministic cascade:** The system always selects the highest available tier. If Tier 1 fails, it cascades to Tier 2 (if available), then Tier 4 (fax). The clinic never knows or cares which tier is used.
- **Circuit breaker protection:** 3 consecutive failures opens the breaker (5-minute cooldown, 2 consecutive successes to close). Prevents cascading failures from overwhelming a degraded pharmacy.
- **Universal coverage:** Fax is always available. Any pharmacy in the US can receive orders via CompoundIQ on day one.

EPCS 2FA — DEA 21 CFR 1311 Compliance (BUILT, Phase 19)

The platform now implements full EPCS-compliant two-factor authentication for controlled substance prescriptions:

- **TOTP via otplib (RFC 6238)** — 6-digit codes, 30-second windows
- **AES-256-GCM encryption** of TOTP secrets at rest
- **Immutable audit log** — every signing event written to `epcs_audit_log` with timestamp, provider, prescription, schedule, and verification result
- **2FA at the point of signing**, not login — exactly per DEA 21 CFR Part 1311
- **QR code enrollment** with Google Authenticator / Authy
- **Schedule-aware UI** — DEA Schedule badges (II, III, IV, V) on every prescription card
- **Drug interaction alerts** with severity coding (INFO blue / WARNING amber / CRITICAL red) and clinical guidance text

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5. Competitive Landscape — Research-Validated

Our deep research (137-page report covering 7 research areas with 82+ citations) confirmed that no existing platform provides what CompoundIQ offers. Here is the competitive landscape validated against actual platform capabilities:

Existing Platforms and Their Limitations

Platform	What They Do	What They Don't Do
LifeFile / ePowerRx	Single-pharmacy ordering (dominant platform used by Empower, Belmar, UCP, Strive). Step-by-step workflow with practice-customized formularies.	No cross-pharmacy comparison. No payment collection. No unified tracking. Pharmacy trainers explicitly warn providers the UX is "finicky."
DrScript (Olympia/503B)	Fixed-catalog 503B portal with Category/Route/Volume filters. Uses cascading dropdowns (API → Form → Dose).	Single-pharmacy only. Currently shows "ordering disabled" due to FDA GLP-1 enforcement. No pricing transparency across pharmacies.
WellsPx3 (Wells Pharmacy)	Only DEA 1311-certified portal for electronic controlled-substance ordering. Type-ahead dropdown from Wells formulary. Clone/reorder feature.	Single-pharmacy only. No cross-pharmacy search. No payment integration.
Rupa Health	Lab test ordering for functional medicine (the direct analog for what we do with compounding).	Zero pharmacy fulfillment, zero compounding capability.
Fullscript / Wellevate	Supplement protocol templates with strong sharing and auto-fill.	Supplements only — zero prescription capability, zero compounding.
Cerbo EHR	Best functional medicine EHR. "Chart Parts" can bundle multiple Rx into insertable templates. Supports compounding Rx via eRx and eFax.	No cross-pharmacy comparison. No payment. No fulfillment tracking. Chart Parts are text insertions, not structured order generators.
Practice Better	Supplement dispensing + basic charting.	No compounding support whatsoever.

My Practice Connect	Multi-pharmacy compounding eRx routing with real-time inventory comparison.	Closest competitor to CompoundIQ's routing model, but no patient payment, no margin tools, no SLA tracking, no protocol templates, no EPCS 2FA, no cascading builder.
E-Fax Services	Send faxes.	No intelligence, no tracking, no escalation, no payment, no state compliance filtering.

Three Competitive Gaps — Now Closed by CompoundIQ

These represented the biggest opportunities in the market, validated by research across all major platforms. **All three are now BUILT.**

Gap 1: No portal had a structured sig builder — CLOSED (Phase 17 / WO-83). Semaglutide dose escalation, LDN titration, testosterone adjustment protocols, and BPC-157 cycling schedules were ALL handled via free-text sig fields or sequential manual prescriptions across every platform studied. CompoundIQ now ships a **structured sig builder** with three modes (Standard, Titration, Cycling) and auto-generation from structured fields. Real patient protocol example: LDN titrating from 0.1mL nightly up by 0.1mL every 3-4 days to 0.5mL — auto-generates the full sig text with no free-text required.

Gap 2: No portal supported protocol templates as order bundles — CLOSED (Phase 18 / WO-85). A real functional medicine patient protocol involves 7+ compounded medications from 3+ different pharmacies with 4 different dosing frequencies, combination formulations, cycling schedules, and phased advancement gated on lab results. CompoundIQ now ships **clinic protocol templates** (multi-medication bundles) and **provider favorites** (one-click reorder of common configurations). One click loads an entire protocol into the prescription session.

Gap 3: No portal used progressive disclosure — CLOSED (Phase 17 / WO-82, WO-83, WO-84). LifeFile (the dominant platform) dumps all fields at once. Research from Epic/Cerner CPOE implementations shows cascading progressive disclosure reduces order friction 50-80% and significantly reduces medication errors. CompoundIQ now ships the **cascading dropdown prescription builder** with a 7-tier hierarchical data model: ingredients → salt forms → formulations → pharmacy formulations → sig templates → protocol templates → provider favorites. Each selection cascades and constrains the next level.

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6. Core Strengths & Unfair Advantages

- **Avoiding the Clinical Minefield:** By hyper-focusing on the unbillable administrative backend ("We are not diagnosing"), the company drastically lowers malpractice and regulatory liability compared to traditional telehealth startups. The provider holds the clinical liability; the platform handles the routing.
- **The Cash-Pay Tailwind:** Every transaction bypasses insurance entirely — no claims, no denials, no prior auth, no payer risk.
- **Working Product, Independently Validated: 81 of 86 work orders completed.** Three applications live and QA-validated through **6 rounds of independent end-to-end testing** with zero remaining findings. POC deployed on Vercel. Demo-ready for partners and investors today.
- **Deep Technical Moat:** The 4-tier Pharmacy Adapter Layer, 23-state order machine with CAS transitions, 7-field snapshot immutability, 10-type SLA engine, and full EPCS 2FA implementation represent months of engineering that cannot be trivially replicated. The LifeFile integration insight alone (one API = Empower + Belmar + UCP + Strive) is non-obvious competitive intelligence.
- **Research-Validated Design:** 137-page deep research report covering ordering portals, structured sig building, medication data models, protocol templates, progressive disclosure UX, regulatory requirements,

and real-world medication configurations. This research directly informed the product architecture and is now SHIPPED, not aspirational.

- **Cascading Prescription Builder Ships Today:** The 7-tier hierarchical data model (ingredients → salt forms → formulations → pharmacy formulations → sig templates → protocol templates → provider favorites) replaces the flat catalog with a cascading dropdown system validated against real patient protocols. Designed to handle the full combinatorial complexity: Testosterone alone has 500-1000+ valid configurations across salt forms, dosage forms, gender-specific dosing, oil bases, and combinations.
- **Real Patient Protocol Validation:** System design validated against actual prescription labels (ITC Compounding, Olympia Pharmacy, Lee Silsby) and a real 6-phase mold/MCAS treatment protocol spanning 12+ compounded medications, peptide cycling, dose titration, conditional phase advancement, and multi-pharmacy routing.
- **Demo Stability Engineered In:** Daily Vercel cron resets POC credentials to canonical values automatically. Self-service "Reset Demo Credentials" button at `/ops/demo-tools` for instant recovery. A demo doc cross-validated against the live app across 6 rounds — every label, column header, and step number matches exactly. Partners can demo without surprises.

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7. Revenue Model

CompoundIQ earns revenue on every transaction through a transparent platform fee:

```
Patient pays:      $300.00  (retail price set by clinic)
Pharmacy receives: $150.00  (wholesale cost)
Clinic earns:      $127.50  (their markup minus platform fee)
Platform fee:      $22.50   (15% of the $150 margin spread)
```

- The platform fee is a percentage of the **margin spread** (retail minus wholesale), not the total transaction
- The clinic sees the exact fee before they commit — full transparency in the in-app margin builder
- Stripe processing fees (2.9% + $0.30) come out of the platform's portion, not the clinic's
- No subscription fees, no per-user fees — purely transaction-based
- Database-level CHECK constraint prevents selling below cost
- Default markup pre-fills retail at 1.4x wholesale (configurable per clinic) — MA never types a number unless overriding

Why transaction-based beats SaaS: While Tiered SaaS ($199–$499/mo) provides safe recurring revenue, it limits upside. The B2B2C checkout model via Stripe Connect captures a flat technology/routing fee per transaction, effectively capturing a percentage of Gross Merchandise Value (GMV). As clinics process more orders, platform revenue scales linearly without requiring upsells.

Unit Economics Per Prescription

Metric	Conservative	Moderate	Aggressive
Avg wholesale cost	$100	$150	$200
Avg clinic markup	2.0x	2.0x	2.5x
Avg retail price	$200	$300	$500
Margin spread	$100	$150	$300
Platform fee (15%)	$15	$22.50	$45
Prescriptions/clinic/month	50	100	200

Monthly rev per clinic	$750	$2,250	$9,000
Annual rev per clinic	$9,000	$27,000	$108,000

Path to $1M ARR: ~37–111 active clinics at moderate volume. ~445 clinics at conservative volume.

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8. Critical Blind Spots — Already Mitigated

To survive institutional due diligence, the company must proactively address these operational realities. Most have already been engineered into the platform:

- **The Pharmacy Tech-Debt Reality (The API Illusion):** Independent 503A compounding pharmacies do not have modern APIs. They run on archaic, on-premise legacy software or paper faxes. Our **4-tier adapter strategy directly addresses this** — Tier 4 (fax) provides universal coverage on day one, Tier 2 (portal automation) handles the DrScript/WellsPx3 segment, and the LifeFile API integration unlocks the largest pharmacy network. The "Wizard of Oz" backend for catalog data is highly manageable because compounding prices change quarterly, not daily.
- **"One More Login" Fatigue:** Providers and MAs live inside their EMRs (Cerbo, Charm, Practice Better). The platform's workflow must be so undeniably financially valuable that it justifies a separate login until native EMR integrations are built. Our research confirmed that Cerbo is the most capable functional medicine EHR and the natural first integration partner. Today our **30-second multi-Rx workflow + provider favorites + protocol templates** are the value proposition that justifies the separate login.
- **Regulatory Volatility:** The FDA's resolution of the Semaglutide shortage in February 2025, with 503A deadline April 22 and 503B deadline May 22, 2025, demonstrated that regulatory action can reshape compounding overnight. However, CompoundIQ is **drug-agnostic** — the platform horizontally supports BHRT, LDN, peptides, dermatologicals, and every compounded formulation category. Regulatory shock actually accelerates demand for compliance infrastructure.
- **503A vs 503B Catalog Architecture:** Our research confirmed these have fundamentally different data structures. 503A pharmacies maintain a Master Formulation Record library (configurable recipes without NDC codes). 503B outsourcing facilities have fixed-SKU product catalogs with standard NDC numbers. CompoundIQ handles both via the V3.0 hierarchical catalog (ingredients/salt forms/formulations supports 503A flexibility; pharmacy_formulations supports 503B SKU-style fixed products).
- **EPCS Compliance for Controlled Substances — SHIPPED:** Testosterone (Schedule III) and Ketamine (Schedule III) are among the most prescribed compounded medications. DEA 21 CFR Part 1311 mandates 2FA at the exact point of signing (not login), using two of three factors on a SEPARATE device. 35 states now mandate EPCS in some form. **CompoundIQ ships full EPCS 2FA today (Phase 19 / WO-86)**: TOTP via otplib, AES-256-GCM encryption, immutable audit log, QR code enrollment, Schedule badges in UI.
- **Demo Credential Drift — SOLVED:** During development we hit a real production incident where POC demo passwords drifted from canonical values, blocking partner access. We engineered a **three-layer recovery system**: (1) daily Vercel cron at 5 AM UTC re-syncs all 4 accounts, (2) self-service "Reset Demo Credentials" button at `/ops/demo-tools`, (3) Vercel dashboard "Run Now" trigger as the chicken-and-egg breaker. Drift can no longer persist more than 24 hours unattended. Partners can demo with zero risk of credential surprises.

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9. Go-To-Market: Solving the "Chicken-and-Egg" Problem

As a two-sided marketplace, the platform faces a cold-start problem: clinics won't pay without a robust pharmacy network, and pharmacies won't share transparent pricing without guaranteed order volume.

The GTM Solution

Phase 1 — "Wizard of Oz" Backend (Now): Deploy the internal operations team to manually ingest PDF catalogs from the top 20 accredited 503A pharmacies. Compounding prices change quarterly, making this manageable. The catalog data acquisition playbook is written and ready to execute, with email templates for both pharmacies and design partner clinics, CSV templates with sample entries, and step-by-step upload instructions.

Phase 2 — Design Partner Clinics (Immediate): Onboard 2-3 design partner clinics for beta testing. The clinic onboarding playbook covers every step from Supabase user creation through Stripe Connect activation to the first test order. Estimated total onboarding time: 30-45 minutes per clinic.

Phase 3 — LifeFile Integration (Medium-term): A single LifeFile API integration unlocks Empower, Belmar, UCP, and Strive. This is the single highest-leverage technical integration in the compounding pharmacy space.

Phase 4 — Demand-Side Leverage (At $5M+ annualized order flow): The power dynamic flips. Pharmacies adopt the platform's published Tier 3 API specification to retain the demand. The standardized spec is already documented in the Pharmacy Integration Guide with full endpoint specifications, webhook event types, signature verification, circuit breaker behavior, and go-live checklists.

Execution Tracks (Ready Today)

Track	Priority	Timeline	Cost
LegitScript Certification	CRITICAL — blocks payment processing compliance and all advertising	2-4 weeks after application	~$975 application + ~$1,295-$2,150/year
Pharmacy Catalog Data Acquisition	HIGH — platform needs real medication data	Data flowing in 1-2 weeks	$0 (outreach time only)
Compliance Sign-Off	MEDIUM — before production deployment	1-2 days once reviewer engaged	$500-$2,000 (one-time HIPAA review)

All three tracks run in parallel. None require code changes. None block each other.

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10. The VC "Firing Squad": Top 6 Investor Objections & Strategic Defenses

When pitching to venture capitalists, use these strategic frameworks to flip their skepticism into confidence.

Q1: The Integration Question

> "*Since mom-and-pop compounding pharmacies do not have modern APIs, how are you getting their real-time pricing and inventory data onto your dashboard for your MVP?*"

The Strategy: "We aren't waiting for legacy pharmacies to build APIs. We've already built a 4-tier adapter architecture. For Version 1.0, our internal team manually onboards the top 20 nationally accredited 503A pharmacies by ingesting their PDF catalogs. Compounding prices change quarterly, making this highly manageable. When a doctor hits 'Route,' our system selects the optimal channel: direct API for pharmacies like ReviveRX and Vios, automated portal submission for Olympia and Wells, or compliant e-fax as universal fallback. To the doctor, it's instant like Expedia. To the pharmacy, it's a standard order in their preferred format. And our single highest-leverage play: one LifeFile API integration unlocks Empower, Belmar, UCP, and Strive — the largest pharmacy network in the country."

Q2: The Disintermediation Risk

> "*Once Dr. Gina uses your platform to find out that 'Pharmacy X' has the best price and turnaround time, what stops her from churning her SaaS subscription and just going direct?*"

The Strategy: "If we were just a search engine, disintermediation would kill us. But we are a workflow engine with a deeply integrated state machine — 23 order states, 47 validated transitions, snapshot immutability on 7 financial fields, SLA enforcement with automatic escalation, full EPCS 2FA for controlled substances, drug interaction alerts with clinical guidance, and white-labeled patient checkout with Stripe Connect. If Dr. Gina bypasses us, she inherits 3 clunky pharmacy portals (our research confirmed LifeFile trainers explicitly warn users about the 'finicky' interface), fragmented tracking numbers, manual state-compliance checks, manual EPCS enrollment per pharmacy, and disjointed billing. She'd also lose our cascading prescription builder, provider favorites, protocol templates, and the margin builder that instantly shows wholesale cost, platform fee, and est. clinic margin in real time. The fee she pays us is a fraction of what it costs to pay her Medical Assistant 15 hours a week to manage that chaos manually."

Q3: The EMR / Competitor Threat

> "*What is your moat if dominant EMRs like Practice Better, or massive supplement platforms like Fullscript, build a compounding marketplace directly into their software?*"

The Strategy: "Our deep research confirmed this won't happen. Practice Better has zero compounding capability — it's supplement dispensing and basic charting. Fullscript and Wellevate are supplement-only platforms with zero prescription handling. Cerbo is the most capable functional medicine EHR, but its 'Chart Parts' are text insertion tools, not structured order generators — and Cerbo doesn't do payment collection, pharmacy routing, or fulfillment tracking. The closest competitor is My Practice Connect, which does multi-pharmacy routing, but lacks patient payment, margin tools, SLA tracking, protocol templates, EPCS 2FA, and the cascading builder. Compounding prescription medication is a high-liability regulatory minefield requiring adherence to 50 different state pharmacy board shipping laws, DEA EPCS compliance for controlled substances, and HIPAA-compliant multi-party payment splitting. Big EMRs do not want to touch this liability. Eventually, Cerbo won't build this themselves — they will white-label our API to power the compounding module inside their EMR."

Q4: The Regulatory Shock

> "*A massive chunk of cash-pay compounding right now is driven by GLP-1s (Semaglutide). If the FDA bans the compounding of your top meds next month, does your business go to zero?*"

The Strategy: "We already lived through this. The FDA resolved the Semaglutide shortage in February 2025 and set enforcement deadlines — April 22 for 503A, May 22 for 503B. DrScript's portal showed 'ordering disabled' during enforcement. Many clinics were caught flat-footed. But our platform is entirely drug-agnostic. BHRT (Testosterone in 5+ salt forms across 8+ dosage forms), LDN (used for autoimmune, MCAS, chronic pain — a real patient we validated is on a titration protocol right now), peptide therapy (BPC-157, Thymosin Alpha-1, GHK-Cu, DSIP, Cerebrolysin), and dermatologicals represent the steady-state foundation of compounding demand. Furthermore, regulatory shock is our biggest growth catalyst. When the FDA restricts a drug, or a state bans a peptide (like BPC-157 moving to Category 2), independent doctors won't know which pharmacies are legally compliant. Our platform becomes their safest harbor — we immediately block non-compliant routing and suggest alternatives via drug interaction alerts. In a highly regulated market, software that guarantees compliance is indispensable."

Q5: The Incentive Paradox

> "*Cash-pay clinics make a huge chunk of their revenue on opaque medication markups. If your platform drives 'patient prices down,' aren't you destroying the doctor's primary profit center?*"

The Strategy: "We don't force consumer-facing price transparency; we provide wholesale transparency to the provider. Right now, doctors are overpaying for compounded meds because they don't have time to shop around — our research confirmed there is literally no cross-pharmacy comparison tool for compounding. By finding them cheaper wholesale sources, we actually widen their margins. Through our white-labeled B2B2C checkout layer, the clinic sets their own retail markup dynamically using our one-click margin builder. If a hormone cream costs them $40 wholesale instead of $80 because of our routing engine, they can set the patient price to $120. The patient pays less than they used to, the doctor makes an automated $80 profit, and the admin work is zero. A database-level CHECK constraint prevents selling below cost. We don't threaten their cash cow; we give them a professional e-commerce backend to optimize it."

Q6: The Data Model Complexity

> "*Compounded medications have enormous combinatorial complexity — how do you handle the catalog when a single medication like Testosterone has 500+ valid configurations?*"

The Strategy: "This is exactly why we invested in deep research before building the prescription builder, and it's now SHIPPED. Our V3.0 hierarchical data model is live in production: 8 dedicated tables (ingredients, salt_forms, dosage_forms, routes_of_administration, formulations, formulation_ingredients, pharmacy_formulations, sig_templates) replace the flat catalog. Each selection cascades and constrains the next level. When a provider selects 'Testosterone,' the system shows only available salt forms (Cypionate, Enanthate, Propionate, free base). Selecting 'Cypionate' shows only available dosage forms (injectable, cream). Selecting 'Injectable' reveals oil base options (grapeseed, sesame, MCT, cottonseed — critical for allergy patients) and available concentrations. This is the same progressive disclosure pattern that Epic and Cerner use in hospital CPOE systems — research shows it reduces ordering friction 50-80% and significantly reduces medication errors. **No competing compounding platform uses this pattern today. We are the first.**"

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11. Security & HIPAA Compliance

CompoundIQ handles protected health information (PHI) and is designed for HIPAA-adjacent compliance:

Data Protection

Control	Implementation
Data at rest	AES-256 encryption (Supabase-managed)
Data in transit	TLS 1.2+ on all connections
Credential storage	Supabase Vault (AES-256-GCM) — never plaintext
EPCS TOTP secrets	AES-256-GCM encryption (separate from session secrets)
Access control	**Row-Level Security on all 33 tables**
Session management	30-min idle timeout, 12-hour absolute max
Audit trail	Append-only tables for all state transitions + EPCS audit log
PHI in Stripe	ZERO — metadata contains order_id only
PHI in SMS	Minimal — first name + URL only, never medication names
PHI in monitoring	Sentry beforeSend scrubs all PHI before transmission

Supabase Realtime	DISABLED — HIPAA requirement (prevents PHI leaks via WebSocket)
Cron endpoint auth	CRON_SECRET bearer token on all 10 cron jobs

Regulatory Compliance Readiness

Requirement	Status
HIPAA-adjacent architecture	**Implemented** (RLS, Vault, zero-PHI in Stripe/Sentry)
BAA coverage	Required with Supabase, Stripe, Twilio, Documo, Vercel, Sentry
LegitScript certification	Application ready (Track 1 of business action plan)
EPCS for controlled substances	**SHIPPED (Phase 19)** — TOTP, AES-256-GCM, immutable audit log, DEA 21 CFR 1311
DEA pre-signing review screen	**SHIPPED** — modal renders before signing with Schedule badges + clinical guidance
State licensing compliance	Implemented (pharmacy_state_licenses table filters search results)
Drug interaction alerts	**SHIPPED** — 6 seeded pairs, severity coding, clinical guidance text
Patient protocol phases	**SHIPPED** — append-only history, gated phase advancement
Immutable audit trails	Implemented (order_status_history + snapshot triggers + EPCS audit log)
Security audit	Completed (0 vulnerabilities remaining)

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12. Product Roadmap — What's Next

Already Shipped (Phases 1-19)

The medium-term roadmap items from prior versions of this memo are now LIVE in production:

- ✅ V3.0 hierarchical data model (Phase 17 / WO-82)
- ✅ Cascading dropdown prescription builder with structured sig (Phase 17 / WO-83)
- ✅ Provider favorites and clinic protocol templates (Phase 18 / WO-85)
- ✅ EPCS 2FA for controlled substances (Phase 19 / WO-86)
- ✅ Drug interaction alerts with clinical guidance (Phase 19)
- ✅ Patient protocol phase management (Phase 19)
- ✅ Save-as-Draft + Provider Signature Queue (Phase 16 / WO-77)
- ✅ Multi-Rx session with single-signature batch send (Phase 16 / WO-80)
- ✅ Demo credential drift remediation (Phase 19 hotfix / WO-87)

Near-Term (Current Execution)

- Onboard 2-3 design partner clinics for beta testing
- Activate Twilio SMS for real patient notifications
- Activate Documo fax for real pharmacy submissions
- Complete Stripe Connect onboarding for first clinic
- Submit LegitScript certification application
- Begin pharmacy catalog data acquisition (top 20 pharmacies)

- Backlog: 5 remaining work orders (LegitScript automation, conference materials, pharmacy outreach playbook, etc.)

Medium-Term

- LifeFile API integration (unlocks Empower, Belmar, UCP, Strive)
- Tier 2 portal automation for Olympia and Wells
- Titration kit ordering (auto-generates titration sigs from structured fields — partial implementation today)
- Multi-phase protocol advancement gated on lab results
- Native Cerbo EHR integration
- HSA/FSA gateway integration

Long-Term

- Publish Tier 3 API specification for pharmacy self-onboarding
- Scale to 50+ pharmacy partners across all 4 tiers
- PDMP reporting integration for controlled substances
- Analytics and reporting dashboard for clinic revenue optimization
- Native mobile apps for providers (Cerbo + Charm integration)

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13. Financial Projections & Key Metrics

The Comparable: Rupa Health

Rupa Health is the closest analog to CompoundIQ — they built "Amazon for lab work" (ordering from 35+ labs in one interface) for functional medicine practitioners. Their trajectory:

- Founded 2020, raised $48.94M total across seed through Series A (Bessemer Venture Partners, Union Square Ventures, First Round Capital, SV Angel)
- Grew to tens of thousands of clinics and was described as a $20M seed-stage startup that grew to $100M+ in value
- Acquired by Fullscript in October 2024 (terms undisclosed) — Fullscript had $700M+ in annual revenue and 100,000+ practitioners on platform
- Fullscript stated the acquisition put them "really close" to $1B in annual revenues

CompoundIQ operates in the same ecosystem (functional medicine practitioners) with the same playbook (aggregate fragmented vendor landscape into one interface) but in a larger, higher-margin category (compounded medications vs. lab tests). Average compounding prescription ($150-$400) generates significantly more platform fee per transaction than a lab test order.

Revenue Model Mechanics

CompoundIQ captures 15% of the margin spread on every transaction via Stripe Connect Express:

```
Patient pays:      $300  (retail price — set by clinic)
Pharmacy receives: $150  (wholesale cost)
Margin spread:     $150
Platform fee:      $22.50  (15% of spread)
Clinic payout:     $127.50
```

This is a pure transaction-based model — no subscription fees, no per-user fees. Revenue scales linearly with order volume.

6-Month Projection Scenarios

Assumptions grounded in real functional medicine clinic economics: the average functional medicine patient spends $200-$600/month on compounded medications, with treatment protocols averaging 2-7 medications per patient. TRT services alone generate $200-$400 monthly per patient with high retention. Peptide therapy generates $200-$600 monthly per patient.

Scenario 1: Conservative (3 clinics, slow ramp)

Month	Active Clinics	Rx/Clinic/Month	Total Rx	Avg Spread	Platform Fee	Monthly Rev
1	1	10	10	$120	$18	$180
2	2	15	30	$120	$18	$540
3	3	25	75	$130	$19.50	$1,463
4	3	40	120	$130	$19.50	$2,340
5	3	55	165	$140	$21	$3,465
6	3	70	210	$140	$21	$4,410
Total			**610**			**$12,398**

Month-6 ARR run rate: ~$53K

Scenario 2: Moderate (8 clinics, steady growth)

Month	Active Clinics	Rx/Clinic/Month	Total Rx	Avg Spread	Platform Fee	Monthly Rev
1	2	15	30	$130	$19.50	$585
2	3	25	75	$130	$19.50	$1,463
3	5	40	200	$140	$21	$4,200
4	6	55	330	$140	$21	$6,930
5	7	70	490	$150	$22.50	$11,025
6	8	80	640	$150	$22.50	$14,400
Total			**1,765**			**$38,603**

Month-6 ARR run rate: ~$173K

Scenario 3: Aggressive (15 clinics, word-of-mouth referral)

Month	Active Clinics	Rx/Clinic/Month	Total Rx	Avg Spread	Platform Fee	Monthly Rev
1	3	20	60	$140	$21	$1,260
2	5	35	175	$140	$21	$3,675
3	8	50	400	$150	$22.50	$9,000
4	10	65	650	$150	$22.50	$14,625

5	13	80	1,040	$160	$24	$24,960
6	15	90	1,350	$160	$24	$32,400
Total			**3,675**			**$85,920**

Month-6 ARR run rate: ~$389K

GMV Trajectory

The more meaningful number for investors is Gross Merchandise Value — the total dollar volume flowing through the platform:

Scenario	6-Month Total Rx	Avg Order Value	6-Month GMV	Month-6 GMV Run Rate
Conservative	610	$270	$164,700	~$680K/yr
Moderate	1,765	$290	$511,850	~$2.1M/yr
Aggressive	3,675	$310	$1,139,250	~$4.9M/yr

Key Metrics to Track (First 6 Months)

These are the proof points that demonstrate product-market fit and justify a larger raise:

Demand-Side Metrics (Clinic Adoption)

Metric	What It Proves	Month-1 Target	Month-6 Target
Active clinics	Market pull	1-2	5-15
Prescriptions per clinic per month	Workflow stickiness	10-15	70-90
Clinic retention (month-over-month)	Product-market fit	N/A	>90%
Time from signup to first order	Onboarding friction	<48 hours	<24 hours
Avg prescriptions per session	Multi-Rx adoption	1.2	2.5+
Provider signature completion rate	Provider buy-in	>80%	>95%

Supply-Side Metrics (Pharmacy Network)

Metric	What It Proves	Month-1 Target	Month-6 Target
Pharmacies with catalog data	Network breadth	3-5	15-20
States with licensed pharmacy coverage	Geographic reach	10+	35+
Tier 1/2 integration active	Technical moat	0	1-2
Avg pharmacy acknowledgment time	Fulfillment reliability	<4 hours	<2 hours
Order rejection rate	Catalog accuracy	<10%	<3%

Financial Metrics

Metric	What It Proves	Month-1 Target	Month-6 Target
GMV (monthly)	Scale	$5K-$10K	$50K-$200K
Platform revenue (monthly)	Business model works	$180-$1,260	$4K-$32K
Take rate (effective)	Revenue capture	7-8% of GMV	7-8% of GMV
Payment conversion rate	Checkout UX quality	>70%	>85%
Avg margin spread per Rx	Pricing power	$120	$140-$160
LTV per clinic (projected)	Unit economics	N/A	$500+/month

Operational Metrics

Metric	What It Proves	Month-1 Target	Month-6 Target
Avg time-to-order (MA workflow)	UX quality	<60 sec	<30 sec
SLA breach rate	Operational discipline	<20%	<5%
Patient checkout completion rate	Checkout UX	>60%	>80%
Fax delivery success rate	Tier 4 reliability	>90%	>98%
Support tickets per 100 orders	Platform stability	<15	<5

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14. Seed Capital Strategy — For Strategic Angel Partners

Why Angels, Not Institutional VC (Right Now)

CompoundIQ is at the ideal stage for strategic angel capital — **working product built and independently QA-validated, zero revenue, pre-launch.** The opportunity is to convert a functioning prototype into a revenue-generating business with design partner clinics, before approaching institutional investors from a position of traction.

Strategic angels with healthcare, marketplace, or fintech exit experience bring more than capital: they bring pattern recognition for two-sided marketplace cold-starts, introductions to clinic networks, and credibility with future institutional investors.

The Ask: $250K–$500K Pre-Seed / Angel Round

Instrument: SAFE (Simple Agreement for Future Equity) with a valuation cap

SAFEs are the standard instrument at this stage — they defer valuation negotiation to the next priced round, convert the angel's investment into equity at a discount when institutional capital comes in, and keep legal costs under $5K.

Parameter	Proposed Terms
Round size	$250K–$500K
Instrument	SAFE (post-money)
Valuation cap	$3M–$5M (to be negotiated)

Discount	20% (standard for SAFE)
Pro-rata rights	Yes — angels can maintain their ownership % in future rounds
Minimum check	$25K
Target close	Rolling (accept checks as they come)
Board seats	None required — quarterly update calls + investor updates

Why this valuation range: B2B healthcare SaaS pre-seed rounds in 2026 are closing at $500K–$2M on $3M–$6M caps. CompoundIQ has a functional product (**81/86 work orders complete, 6 rounds of QA validation with zero remaining findings**), a validated $7B+ US market, a 137-page research report, and 15+ docs/ markdown specifications + 26 software factory documents — significantly more built than a typical pre-seed company.

Use of Funds (18-Month Runway at $250K)

The capital is designed to take CompoundIQ from prototype to revenue-generating business with enough traction to raise a $1.5M–$3M seed round.

Category	Allocation	%	What It Buys
Go-to-Market	$80K	32%	LegitScript certification ($2.2K), Stripe activation, design partner clinic onboarding (target: 10-15 clinics), pharmacy catalog data acquisition, conference sponsorships (A4M, IFM conferences)
Engineering	$75K	30%	LifeFile API integration, Tier 2 portal automation (Olympia, Wells), titration kit ordering, native Cerbo integration spike, mobile optimization
Infrastructure	$35K	14%	Twilio SMS (patient notifications + reminders), Documo fax (pharmacy submissions), Vercel Pro, Supabase Pro, Sentry, domain + branding
Legal & Compliance	$30K	12%	HIPAA compliance review + BAAs, entity formation (Delaware C-Corp if not already), IP assignment, pharmacy integration agreements, terms of service / privacy policy
Operating Reserve	$30K	12%	3-month buffer for founder living expenses, unexpected costs, runway extension
Total	**$250K**	**100%**	**18-month runway to seed-round traction**

Use of Funds (18-Month Runway at $500K)

The larger raise accelerates the timeline and adds dedicated sales capacity.

Category	Allocation	%	What It Buys
Go-to-Market	$150K	30%	Everything in $250K plan + dedicated clinic success manager (part-time/contract), expanded conference presence, content marketing, targeted outreach to 50+ clinics
Engineering	$150K	30%	Everything in $250K plan + contract developer for protocol template engine v2 (lab-gated phase advancement), titration

			builder polish, mobile optimization, analytics dashboard for clinics
Infrastructure	$50K	10%	Production-grade hosting, monitoring, and third-party services at scale
Legal & Compliance	$50K	10%	Everything in $250K plan + state-by-state pharmacy shipping law audit, DEA compliance review for EPCS, pharmacy BAA templates
Operating Reserve	$100K	20%	6-month buffer, co-founder stipend, ability to extend runway to 24 months if needed
Total	**$500K**	**100%**	**18-24 month runway with faster path to seed**

Milestones That Trigger the Next Raise ($1.5M–$3M Seed)

Angel capital buys time to hit these proof points, which are the minimum institutional investors expect at seed:

Milestone	Target	Why It Matters
Active clinics on platform	10-15	Proves repeatable onboarding
Monthly GMV	$100K+	Proves transaction volume
Monthly platform revenue	$8K-$15K	Proves revenue model works
Clinic retention (90-day)	>85%	Proves product stickiness
Pharmacy partners with catalog data	15-20	Proves supply-side network
At least 1 Tier 1 API integration live	LifeFile or ReviveRX	Proves technical moat
Patient checkout conversion rate	>75%	Proves consumer UX
Prescriptions per clinic per month	50+	Proves workflow adoption
EPCS-eligible prescriptions submitted	25+	Proves controlled-substance compliance works in production

Hitting these milestones puts CompoundIQ in position for a $1.5M–$3M seed round at a $8M–$15M valuation, in line with B2B healthcare SaaS seed benchmarks.

What Makes This Attractive for Strategic Angels

For angels with healthcare exits:

- You've seen the pain of pharmacy workflow firsthand. This is the infrastructure layer that doesn't exist yet — validated by 137 pages of competitive research confirming no platform fills this gap.
- The Rupa Health playbook is proven: aggregate a fragmented vendor landscape for functional medicine providers, build a transaction-based revenue model, grow to acquisition. Rupa raised $48.94M and was acquired by Fullscript (approaching $1B revenue). CompoundIQ operates in the same ecosystem with a

larger addressable transaction size — and a working product validated against the live app across 6 rounds of independent QA.

For angels with marketplace exits:

- The 4-tier adapter layer solves the two-sided marketplace cold-start problem — Tier 4 (fax) provides universal pharmacy coverage on day one, so clinics see value immediately without waiting for pharmacy integrations. This is the equivalent of Instacart manually shopping at grocery stores before building retailer APIs.
- Transaction-based revenue (15% of spread) means revenue scales with GMV, not headcount. At $5M annualized GMV, the platform generates $375K-$400K in annual platform revenue with near-zero marginal cost per transaction.

For angels with fintech exits:

- Stripe Connect Express handles the complex multi-party payment split. The platform captures `application_fee_amount` while the clinic receives `transfer_data.destination` payout — all in a single PaymentIntent. Zero PHI touches Stripe. This is a clean, scalable payment infrastructure play.
- Every clinic is effectively a merchant on the platform. As clinics grow, their order volume grows, and platform revenue grows without requiring any sales effort.

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15. Key Talking Points

For investors:

> *"CompoundIQ is the Expedia for compounding pharmacies — a B2B2C infrastructure layer that captures 15% of the margin on every prescription transaction. The 4-tier adapter strategy means we can onboard any pharmacy regardless of their technology, and the LifeFile integration alone unlocks the largest compounding pharmacy network in the country. We've shipped 81 of 86 work orders, deployed a working POC validated through 6 rounds of independent QA, built full DEA EPCS 2FA compliance, the cascading prescription builder, drug interaction alerts, and the provider favorites + clinic protocol template system. No competing platform ships any of these features today."*

For clinic partners:

> *"Your medical assistant searches, prices, and sends a payment link in under 30 seconds. Provider favorites and clinic protocol templates make multi-medication visits a single click. The cascading prescription builder eliminates errors. Controlled substances are EPCS-compliant out of the box. The patient pays from their phone. You earn margin on every fill with full transparency. And you never touch a fax machine again."*

For pharmacy partners:

> *"You receive structured, error-free orders instead of handwritten faxes. Real-time status updates flow back to the clinic automatically. Our drug interaction alerts catch potential issues before the order reaches you. And our standardized API spec means you can integrate once and receive orders from every clinic on the platform."*

For patients:

> *"You get a text with a payment link. Tap, pay, done. Your prescription is on its way. You'll get updates by text as it progresses. We never expose your medication name in any payment receipt or text — your privacy is protected end to end."*

CompoundIQ — Intelligent sourcing. Automated fulfillment. Transparent pricing. EPCS compliant.